جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : مرفت محمد حماد -
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** ---
رقم البطاقة : 26610210101625 **رقم التليفون :** 01003961539 **تاريخ تسجيل الإستمارة :** 15-10-2026
العمر(تاريخ الميلاد) : 21-10-1966 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبطات الاروماتيز [14161] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مرفت محمد حماد -
National ID: 26610210101625

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Second Line Hormonal Treatment Aromatase Inhibitor**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
مرفت
محمد حماد هلال
٢٧ ح شاكر ش الداوديه
الدرب الاحمر - القاهره
٢٩٠١٦٠١٢١٠٢٦٦
LV3627312
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٢٩٦١٠٢١٠١٠١٦٢٥
٢٠٢٦/٠٧
ربة منزل
انثى مسلمة متزوجة
ناصر محمد يوسف على
البطاقة سارية حتى ٢٠٣٣/٠٧/٠٤
١
National ID – Back
MOCTYPE ID_BACK

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

08/13/2026

MDT FORM

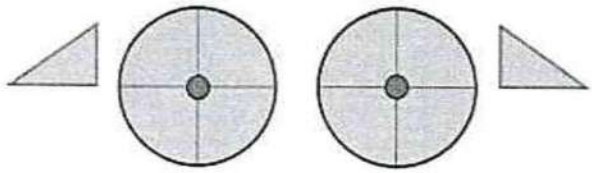
Name:	مرفت محمد حماد هلال	**MRN:**	11437
Age:	60 Y	**ID:**	26610210101625
Menopausal Sta		**Phone:**	01101258642
NRS SCORE:		**Marital Status:**	married
BMI:		**Family History:**	-ve
CO Morbidity:	NAD	**Reason Of MDT:**	NEW

Clinical Data:

BX: IDC G 11

ER: 8/8 **PR:** 5/8 **HER2:** 1 **KI67:** 15

R L



Sonomammography:
LT UIQ mass 4x3x4 cm 16 cm from nipple

Mets Workup:
free

History:

Recommendations:
for possibility of lt WLE +SLNB

TNM

MDT Outcome & Plan
UPFRONT LT WLE +SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د لدى حسن	د محمد نبراوي				

MDT Form
[[DOCTYPE:MDT_FORM]]

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترنشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	مرفت محمد حماد هلال	**Case No.**	**276292**
Sex/Age	**Female 59 Y**	**Patient No.**	**20706**
Registered Date	**13/6/2026 11:08:38 AM**	**Referred By Dr.**	**Out Patient**
Validation Date	**23/6/2026 12:29:22 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**30/6/2026 10:35:55 AM**	**Pathology No.**	**SR-694/26**

Clinical Data: Left breast mass (BIRADS IVc).

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:
One referred block labelled by (26010547) and five referred slides including one H&E stained slide and four immune stained slides for (ER, PR, HER2 & KI67).

Microscopic examination:
-Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 1).
- Ductal carcinoma in situ: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 99% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 1% of tumor cells (score 5/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 15%.

Conclusion:
Left breast mass, Referred block & slides (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 5/8).

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
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Reporting Date	30/6/2026 10:35:55 AM	**Pathology No.**	SR-694/26

- Her2: Low (score 1+).
- Ki67%: 15%.

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
***** Scoring system: "Her2"***
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim.

جوستاف روسي

Patient's Name: Mervat Mohammed Hamad Hilal
Date: 2026-08-06
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Left breast mass lesion measuring 56 x 47 mm showed heterogenous enhancement
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- **Left breast mass lesion**
- **No hepatic or pulmonary metastatic lesions.**

Dr.Asser Ahmed

Dr.Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]

Name: Mrs. Mervat Mohamed Hammad Helal

Date: 8/4/2026 ***ID:6226000052*** ***Acc.:316023087***

Referring Prof. Dr. Mahmoud Hassan

Digital 3D soft tissue mammography & tomosynthesis with ultrasound examination of both breasts, revealed:

Indication: left breast palpable lump.

- (ACR.b).
- Left breast upper posterior third of the breast lobulated dense lesion is seen resting/invading the underlying chest wall muscles. The tomosynthesis slices more clarifying the lobulation.
- Normal right breast parenchyma with no evidences of dense spiculated masses or architecture distortion seen.
- Right UOQ small partially well circumscribed partially obscured isodense lesion. The tomosynthesis slices more clarifying the margins.
- No suspicious micro-calcific clusters noted.
- Normal skin thickness and contour of both sides.
- Normal areola / nipple complexes on both sides.

Ultrasonographic examination showed:

- Left breast shows upper inner quadrant large lobulated heterogenous hypoechoic mass lesion with surrounded desmoplastic reaction and mild internal vascularity by color Doppler, it seen invading the underlying pectoralis muscle and about 3 mm from the overlying skin, it measures about 4.5 x 3.2 x 3.9 cm at 11 o'clock position with distance about 16.7 cm from the nipple (corresponding to the left breast lump).
- Right breast shows upper outer small well defined hypoechoic lesion at 10 o'clock position measures about 4.7 x 2.3 mm, could be small cyst verses small intramammary lymph node.
- No retro-areolar dilated ducts noted.
- Normal skin thickness and contour.
- Normal nipple/areola complexes.
- Bilateral non-specific axillary lymph nodes with preserved oval shaped and central echogenic hilum.

Opinion:

- Left breast UIQ highly suspicious mass lesion as described (BIRADS 4c).
- Right breast UOQ small benign looking lesion as described, could be small cyst verses small intramammary lymph node (BIRADS 2).

Recommendation:

- Ultrasound guided biopsy from the left breast mass.

Truly yours,
Dr. Ayat Nadi, FEBR, ABR

*Printed:***wa.** *09/04/26 1:16:20 PM*

Mammogram
[DOCTYPE:MAMMOGRAM]

Name: **Mrs. Mervat Mohamed Hammad Helal**

BI-RADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BI-RADS 0: Incomplete - need additional imaging evaluation or comparison with previous films.
- BI-RADS 1: Normal examination - nothing to comment on - Routine mammography screening is recommended.
- BI-RADS 2: Benign findings - routine mammography screening is recommended.
- BI-RADS 3: Probably benign <2% malignancy - short interval follow-up is suggested.
- BI-RADS 4: Suspicious >2 but <95% - Biopsy should be considered.
  - 4A: low suspicion of malignancy: 2-20%.
  - 4B: Mod suspicion of malignancy: 10-50%.
  - 4C: High suspicion of malignancy: 50-90%.
- BI-RADS 5: Highly suspicious of malignancy >95% - appropriate action should be taken.
- BI-RADS 6: Known - biopsy proven malignancy. Surgical excision when clinically appropriate.

American college of radiology (ACR) scoring of breast density:

- ACR. a: almost entirely fatty.
- ACR. b: scattered areas of fibro-glandular density.
- ACR. c: heterogenously dense.
- ACR. d: extremely dense.

GUSTAVE ROUSSY INTERNATIONAL

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جوستاف روسي انترناشونال – مصر معمل الباثولوجي

جوستاف روسي

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- Ki67%: 15%.

Conclusion:
Left breast mass, Referred block & slides (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 5/8).

CAP ACCREDITED

[[DOCTYPE:PATHOLOGY REPORT]]
Pathology Report

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Signature
Dr. Walaa Ibrahim Ghoneim.

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية هتمم صحة مصر

10/07/2026

MDT FORM

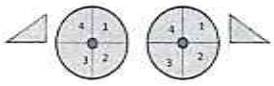
Name:	مرفت محمد حماد هلال	**MRN:**	11437
Age:	60 Y	**ID:**	26610210101625
Menopausal Sta	post	**Phone:***	01101258642
NRS SCORE:		**Marital Status:**	M
BMI:	0	**Family History:**	Free
Gender			
CO Morbidity:		**Reason Of MDT:**	Re MDT
Surgical History:*	Cs once	**ECOG Performance Status**	○ 0 ○ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX: IDC G2

ER: 8/8 **PR:** 5/8 **HER2:** +1 **KI67:** 15%

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T N M

History:

New case, 1st diagnosed at 04/2026 didn't receive any treatment. Now cT4b N1 with skin edema and pedueorange

Sonomammography:

New Mammogram requested

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Biomarker/Molecular

Mets Workup:

Aug 06, 2026: Ct CAP free

Clinical Staging:

Recommendations:

Start NACT until new mammography done, then Lt mastectomy & axilla according to response.

Pathological Staging:

MDT Outcome & Plan

In view of having cT4b N1 disease.....for:

1.New mammosonography

2.Bone scan

3.Meanwhile, start Et (AI) with close monitoring of response.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د شيماء لاشين	د امنيه عيسوي		

7/10/2026

7/10/2026